INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pertuzumab + trastuzumab maintenance

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00726
Version 3a

Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.

Treatment stages

01 **A 21-day reference block**
Treatment → checks → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

HER2-positive metastatic breast cancer · maintenance

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

This maintenance treatment continues HER2 blockade to help control metastatic or unresectable recurrent HER2-positive breast cancer after chemotherapy.

HER2-TARGETED ANTIBODY

Pertuzumab



Blocks HER2 pairing with other receptors

Pertuzumab binds a different area of HER2 from trastuzumab. It limits receptor pairing, so the two antibodies act together at different points on the same growth pathway.

HER2 pairing → Pertuzumab blocks pairing → Growth signals reduced

What to watch for

Report diarrhoea early, especially with poor fluid intake. Keep cardiac checks and report infusion symptoms immediately.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Attend planned echoes. Report breathlessness, ankle swelling, rapid weight gain or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

The chemotherapy part of your earlier combination has finished. Pertuzumab and trastuzumab continue to target HER2 while the team reviews cancer control and treatment tolerance. Maintenance remains active treatment.

Why does this phase feel different?

This card contains the two HER2 antibodies and no routine cytotoxic chemotherapy. Heart checks, infusion-reaction precautions and attention to diarrhoea remain important. This metastatic maintenance pathway has no fixed one-year stopping point; it is different from treatment after surgery for early breast cancer.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

Your treatment aim

Confirm the setting and the plan.

The larger pathway

Pertuzumab and trastuzumab continue after the chemotherapy part of the metastatic treatment plan. Maintenance remains active treatment, with response and heart checks.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Where this treatment fits

Pertuzumab + trastuzumab maintenance

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TREATMENT → CHECKS → REVIEW

A 21-day reference block

D1 D21

Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.

A general pathway, not an appointment calendar. Your team confirms the order, overlap and duration; no date entry is needed.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle numbers are examples for uninterrupted three-weekly treatment. Schedule checks by elapsed time and clinical need. Moving to maintenance does not restart the cardiac-monitoring clock. The clinical team books and reviews results; this map does not track completed tests.

Check	Before this phase	Before every 21-day cycle	Week 12 · around C5	Weeks 24 / 36 · around C9 / C13	Later / treatment change
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Review baseline / latest cardiac assessment and the next due date	Review symptoms; echo is not automatically needed before every infusion	Repeat at 12 weeks; earlier if symptoms or impairment	Continue every 12 weeks; preserve the existing due date	NCCP 12-weekly default while treatment continues; a different interval needs an explicit specialist plan.
Blood tests	Baseline full blood count (FBC), renal and liver profile	FBC, renal and liver tests before every cycle	Continue pre-cycle blood tests	Continue pre-cycle blood tests	Follow abnormalities and the next treatment plan

Check	Before this phase	Before every 21-day cycle	Week 12 · around C5	Weeks 24 / 36 · around C9 / C13	Later / treatment change
Cancer imaging / markers	Record disease sites and the comparison scan	Review new symptoms at every visit	Metastatic: response scans when due under the individual plan	Individual scan interval; markers only if informative	Metastatic scans commonly every 2–4 months; not an NCCP fixed-cycle rule
electrocardiogram (ECG) / bone health	ECG when indicated by cardiac assessment; dual-energy X-ray absorptiometry (bone density) (DEXA) only if endocrine / fracture risk indicates	No automatic every-cycle ECG or DEXA	Risk-based clinical plan	Risk-based clinical plan	Keep any separately prescribed endocrine and bone-health follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00726 v3a · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · response assessment

THE WHOLE TREATMENT JOURNEY

HER2 maintenance: treatment and checks continue

Maintenance · HER2-positive metastatic / unresectable recurrent breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Earlier combination	Maintenance transition	Every 3 weeks	Regular response review	Beyond year 1
Cancer medicines NCCP 00726	Chemotherapy with HER2 treatment	Review benefit, tolerance and last dose date	Pertuzumab + trastuzumab infusions	Continue while benefit and tolerance support the plan	Ongoing treatment; no fixed one-year stopping rule
Local / endocrine care METASTATIC · EARLY	Prior surgery or radiation varies	Discuss symptom-directed local treatment	Radiotherapy only if separately indicated	Endocrine therapy may be considered if HR-positive	multidisciplinary team (MDT) agrees any changes

Care / check	Earlier combination	Maintenance transition	Every 3 weeks	Regular response review	Beyond year 1
Blood tests NCCP 00726	Carry previous results forward	Blood count, kidney and liver tests	Before each cycle	Additional tests when indicated	Continue according to the active regimen
Heart ultrasound NCCP 00726 · ESC	Count from first HER2 exposure	Check latest echo and due interval	Symptoms reviewed every visit	NCCP: echo every 12 weeks	12-weekly default; interval changes require cardiac risk review
Cancer scans METASTATIC	Baseline extent documented	Use prior scans as comparison	Report new symptoms between scans	Usually every 2–4 months; modality / interval individualised	Keep a documented response-assessment plan
Tumour markers MARKERS	Consider if informative	Confirm whether used for this person	No universal marker schedule	Adjunct to symptoms and scans, never alone	Continue only when clinically useful

What each check is for

1

Heart ultrasound

NCCP: baseline and every 12 weeks during HER2 treatment

Checks heart function while treatment continues.

Ask / act: Ask when the next echo is due; a move to maintenance does not restart the clock.

2

Response imaging

Usually every 2–4 months; earlier if clinically needed

A baseline scan records where the cancer is. Response is usually reviewed every 2–4 months, adjusted for the treatment, disease pace, symptoms and previous results. computed tomography (CT) chest/abdomen/pelvis is common when appropriate; the team chooses the imaging needed for the known sites.

Ask / act: Ask what scan is planned, when results will be discussed and who to contact for new symptoms.

3

Tumour markers, if useful

Only if included in the individual monitoring plan

CA 15-3 or carcinoembryonic antigen (CEA) may be helpful in selected metastatic cases, especially if previously raised. They support the assessment with symptoms and imaging; a marker alone does not decide whether treatment is working or should change.

Ask / **act:** Ask what a change means alongside the scan and clinical review.

4



Missed or delayed infusion

Whenever a treatment gap occurs

A long gap can change the required antibody loading doses.

Ask / **act:** Contact the team to confirm the new prescription; do not assume the old visit plan still applies.

5

Separate early-breast pathways

At every transition

This particular NCCP code is for metastatic/unresectable recurrent maintenance.

Ask / **act:** Ask the team to confirm the setting and aim on the written plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.



STEP 1

Before the first visit

Before maintenance, review blood tests, the latest heart assessment, prior treatment response, last antibody doses and the next echo due date.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Check treatment tolerance and symptoms, then receive the prescribed pertuzumab and trastuzumab infusions with observation. Premedicines depend on previous reactions and the individual plan; they are not automatic chemotherapy premedication.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

The visit includes review, infusions and observation. Maintenance does not automatically require new loading doses; an interval of 6 weeks or longer needs review of both antibody loading schedules. Ask your team for the expected visit time.

Before the first treatment

Checks and preparation matter.

Heart ultrasound

NCCP: baseline and every 12 weeks during HER2 treatment. Checks heart function while treatment continues. Ask when the next echo is due; a move to maintenance does not restart the clock.

Response imaging

Usually every 2–4 months; earlier if clinically needed. A baseline scan records where the cancer is. Response is usually reviewed every 2–4 months, adjusted for the treatment, disease pace, symptoms and previous results. CT chest/abdomen/pelvis is common when appropriate; the team chooses the imaging needed for the known sites. Ask what scan is planned, when results will be discussed and who to contact for new symptoms.

When treatment changes or finishes

The next phase has its own checks.

Know the continuing plan

This particular NCCP code is for metastatic/unresectable recurrent maintenance. Ask the team to confirm the setting and aim on the written plan.

Keep reporting symptoms

New or lasting symptoms need assessment between appointments and after treatment stops. Ask for a written summary of continuing medicines, follow-up tests and the contact plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Pertuzumab + trastuzumab	 **Day 1** intravenous (IV)
Infusion reaction	 **Day 1** Staff observe
Diarrhoea	 **Days 1–21** Report changes promptly
Cardiac symptoms	 **Days 1–21** Any day; keep the echo schedule

Treatment days: Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.

The bigger picture: Use the whole-treatment journey for surgery, radiation, scans and longer-term checks.

Read the phase labels: The prescription and team's instructions determine your actual doses and any interruptions.

Why these bands stay broad: Treatment bands show the source schedule. Symptom bands show possible patterns, not measured probabilities; serious effects can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00726 v3a · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

Between-treatment notes

Day 1–21: Follow the phase and prescribed schedule

Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course. Keep planned tests and call for symptoms at any time.

Support around the medicines

1 **Infusion planning**

Confirm the last treatment date with the team; long gaps can change the loading-dose plan.

2 **Heart checks**

Echoes continue on maintenance. Report new breathlessness, ankle swelling or rapid weight gain.

3 **Bowel symptoms**

Report diarrhoea early and follow the agreed hydration/support plan.

4 **Response and daily life**

Keep response-review appointments. Ask for support with fatigue, activity, sexual wellbeing and practical concerns.

Living with treatment

Ask for support that fits your priorities.

Infusion planning

Questions worth asking

Do I need to enter dates?

No. Both charts explain the expected sequence and general intervals without any dates or personal information. Your hospital arranges actual appointments.

Which tests should I expect?

The treatment journey lists baseline and ongoing checks, why they matter and what to ask. It does not mark tests as booked or completed.

What happens if treatment is delayed?

Your team may delay, reduce, change or stop a medicine. Ask which component changes and what happens to the next review; do not restart independently.

What about fertility, pregnancy or breastfeeding?

Discuss fertility preservation and reproductive plans before treatment. The team should explain contraception and breastfeeding restrictions for every medicine, including how long precautions continue afterwards.

How should I use trial numbers?

Trial results describe groups with defined eligibility. Ask how well those patients and their treatment match your situation; the figures are not an individual prediction.

Why was this treatment chosen for me?

The chemotherapy part of your earlier combination has finished. Pertuzumab and trastuzumab continue to target HER2 while the team reviews cancer control and treatment tolerance. Maintenance remains active treatment. This card contains the two HER2 antibodies and no routine cytotoxic chemotherapy. Heart checks, infusion-reaction precautions and attention to diarrhoea remain important. This metastatic maintenance pathway has no fixed one-year stopping point; it is different from treatment after surgery for early breast cancer.

Try explaining the plan back: Can you explain the current phase, which checks are expected next, and which symptoms mean you should call today? You do not need to enter dates to read the plan.

Pertuzumab

- Diarrhoea and dehydration
- Infusion / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pertuzumab + trastuzumab maintenance

Active medicines: Pertuzumab + Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine prophylactic antiemetic, corticosteroid or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is added for these antibodies alone. Review individual symptoms and previous infusion reactions.
- Confirm the last administration date. A gap of 6 weeks or longer requires review of both antibody loading doses under the NCCP schedule.
- Keep the existing cardiac-monitoring clock when moving to maintenance; review new breathlessness, swelling, fatigue or rapid weight gain.
- Report diarrhoea early; agree hydration, symptom treatment and when to contact the unit.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Pertuzumab + trastuzumab maintenance

Pertuzumab and trastuzumab are given by infusion every 21 days after the chemotherapy phase of the metastatic treatment plan. Duration depends on benefit, tolerance and the agreed plan; this is not a fixed one-year adjuvant course.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart pumping function

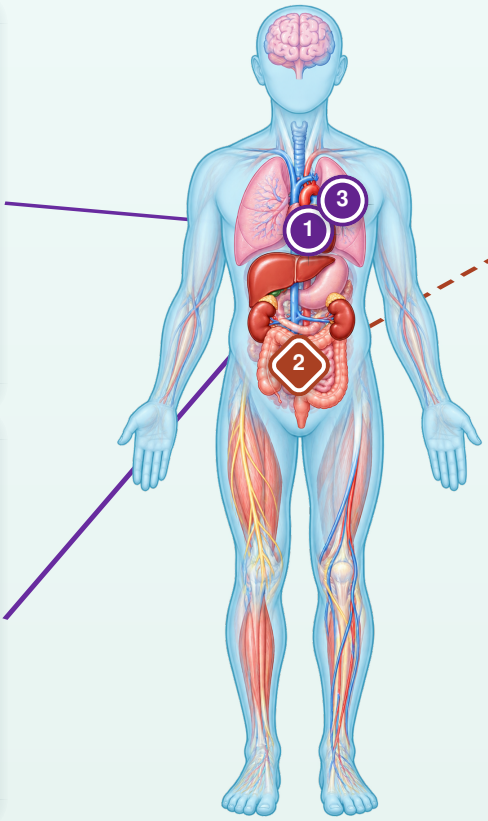
Pertuzumab + trastuzumab

Keep scheduled echoes. New breathlessness, ankle swelling or reduced exercise tolerance: contact oncology promptly.

3 Infusion / allergic reaction

Both HER2 antibodies

During treatment, tell staff immediately about flushing, rash, chest tightness or breathing difficulty.



2 Diarrhoea

Especially pertuzumab

Report frequent / watery stools early, especially with dizziness or poor fluid intake.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Draft — independent clinical, pharmacy and patient/PPI review pending

Cycle number: _____ Start date: _____ Next review: _____

Pertuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Trastuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Pertuzumab + trastuzumab maintenance

NCCP 00726 v3a · 21-day cycle

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1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00726/ · no patient information.

Draft for review · SACTCheck v0.84.14

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00726 v3a · official regimen ↗**
   https://healthservice.hse.ie/documents/6599/726_v3a_Pertuzumab_trastuzumab.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NCCP antiemetics v8, Table 1 pp4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf